



Veterans Administration

VHA FAX TRANSMITTAL

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**Audie L. Murphy VA Hospital,
STVHCS**

7400 Merton Minter
San Antonio, TX 78229
210-617-5300

DATE:	Apr. 23, 2024
ATTN:	
FROM:	Community Care
SUBJECT:	Referral

Note:

Good afternoon,
Please see referral for review and scheduling
Thank you



U.S. Department
of Veterans Affairs

**VA Form 10-7080 - Approved Referral For
Medical Care**

Veteran Name: Dockins, Floyd Norman

Veteran ICN: 1018598065V669163

Veteran EDIPI: 1413316681

Veteran Date of Birth: 1950-01-27

Veteran Preferred Name:

Pronoun:

Pronoun Description:

Veteran Address: 38 TERRY STREET
PORT LAVACA, TX 77979

Veteran Phone Number: (928)830-3524

Veteran Mobile Phone Number (if Known): (928)830-3524

Veteran Business Phone Number (If Known):

Veteran Email Address (If Known): CYNTHIADOCKINS@YAHOO.COM

Referral Number: VA0036934918

Priority: Routine

Referral Issue Date: 2024-04-04

Expiration Date: PRELIMINARY 2024-10-01 (SEE BELOW)*

First Appointment Date: SUPPLY TO VA ASAP

Referring VA Facility: Audie L. Murphy Memorial Veterans' Hospital

VA Telephone Number: 210-949-3850

VA Fax Number:

Initial Community Care Provider/Facility: VICTORIA DERMATOLOGY PA

Initial Provider Location: VICTORIA DERMATOLOGY PA-115 Medical Dr ; Ste 202, Victoria, TX, 77904-207N00000X

Provider Name (if known): PATEL, AMIT

Provider Phone: 281-836-4690

Provider Fax: 409-729-2449

Community Provider NPI: 1346553120

Caregiver Type:

Caregiver Details:

***IN ORDER TO PROVIDE THE FULL DURATION OF THE STANDARDIZED EPISODE OF CARE (SEOC)
PLEASE CONTACT THE REFERRING VA FACILITY WITH A FIRST APPOINTMENT TO CALCULATE
A FINAL EXPIRATION DATE. CLAIM PAYMENTS ARE DEPENDENT UPON THESE DATES.**

Any claim related to this episode of care **MUST INCLUDE THE APPROVED REFERRAL NUMBER** as the
Referral Number or Prior Authorization number.

**Please see below for Additional VA Referring Facility Information, Billing Information
and Appointment/Provider Information.**

Pertinent Clinical Information

**Please view the Clinical Information in the VA Order section for more information related to the Original VA Order
Reason for Request.**

Chief Complaint: He does have recurrent skin cancer. He was diagnosed with Basosquamous carcinoma of skin lower
extremity several years ago. I believe he has the same 1 going on on his lower abdomen. Consult community care
dermatology, preferably in Victoria versus El Campo Texas. Mass

Patient History / Clinical Findings / Diagnosis (Co-Morbidities): n/a Is this referral for Service Connected condition? No Per VA PBM Formulary Process Management Policy from VHA Handbook 1108.08, the use of drugs solely to improve normal physiologic function or to enhance body appearance is generally not considered medically necessary and therefore are not to be prescribed. Medications used for this purpose are considered cosmetic. The use of medications for hair re-growth (e.g., minoxidil, finasteride, or pimecrolimus) are considered cosmetic in nature and should not be prescribed unless care is being provided in connection with the treatment of a service- connected injury.

Provisional Diagnosis: R222 Localized swelling, mass and lump, trunk

Services Authorized

The VA Order Reason for Request is the official clinical order. This scope of services associated with the medical care for this authorization is found below. Necessary services that are not included must be requested using the Request for Services procedures. Please visit the VHA Storefront www.va.gov/COMMUNITYCARE/providers/index.asp for additional resources and requirements.

Service Requested: Dermatology_PRCT SEOC 1.0.12

Category of Care: DERMATOLOGY

Procedural Overview - Standardized Episode of Care (SEOC)

Dermatology_PRCT SEOC 1.0.12 Duration: 180 Days

Description:

This authorization covers services associated with the specialty(s) identified for this episode of care, including all medical care listed below relevant to the referred care specified on the consult/referral order.

NOTE: Please note this SEOC does not cover cosmetic procedures that are not considered medically necessary or correct a functional disability as they are excluded from the Medical Benefits Package.

NOTE: This SEOC does not cover Mohs. A separate referral specifically for Mohs with the Mohs SEOC is required.

NOTE: Superficial Radiation Therapy (SRT) is not covered in this episode of care. SRT is only to be performed and approved by a radiation oncologist and not a dermatologist. SRT must be requested through an RFS and approved using one of the radiation therapy SEOCs.

NOTE: Additional consultations needed relevant to the patient complaint/condition require VA review and approval including a referral to another specialty for secondary/complex closure.

NOTE: This SEOC does not cover the removal of skin tags, seborrheic keratosis, and other benign skin lesions unless they are bleeding, painful, or changing.

No.	Service/Procedure	Number Of Visits Authorized
1	Initial outpatient evaluation (including full body exam), treatment, and follow-up visits for the referred condition on the consult/referral order	999
2	Diagnostic imaging relevant to the referred condition on the consult/referral order	999

3	Labs and pathology relevant to the referred condition on the consult/referral order	999
4	Procedures performed by the dermatology provider relevant to the referred condition on the consult/referral order including but not limited to: cryotherapy, biopsy (e.g., excisional, shave, punch), excision, repair, excimer laser, photodynamic therapy, UVB therapy, debridement, medically necessary hair removal	999

Additional Information:

* Please visit the VHA Storefront www.va.gov/COMMUNITYCARE/providers/index.asp for additional resources and requirements pertaining to the following:

* Pharmacy prescribing requirements

* Durable Medical Equipment (DME), Prosthetics, and Orthotics prescribing requirements

* Precertification (PRCT) process requirements

* Request for Services (RFS) requirements

REFER ALL QUESTIONS RELATED TO THIS APPROVAL TO THE ISSUING VA OFFICE

Referring VA Facility: Audie L. Murphy Memorial Veterans' Hospital

Station Number: 671

Telephone Number: 210-949-3850

Address: 7400 Merton Minter Boulevard SAN ANTONIO TX 78229

Referring Provider: HOHLT, RUSSELL

Referring Provider NPI: 1497988109

Unique Consult No: 671_8545155

Program Authority: Authorized/Pre-authorized VA Referral (not otherwise specified) - 1703

Affiliation: Triwest

Network: CC Network 4

Appointments/Providers Assigned to the Referral

Provider/Facility Name	Provider/Facility Location	Appt Date	Appt Time	Telephone #	Fax #
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Additional Service Information**Request for Services (RFS)**

A Request for Services (RFS) is a provider-generated request for new or additional care outside the scope of the current approved referral/authorization. Provider should always submit the RFS directly to the authorizing VAMC, preferably via the HSRM portal. Providers should always submit an RFS on the same day it is determined it's needed and before delivering care, unless it is emergent care. In that case, the RFS can be submitted simultaneously.

How to Submit an RFS

VA prefers providers submit an RFS via the HSRM portal, available on the VA's website

- Go to the VA Storefront at <https://www.va.gov/COMMUNITYCARE/providers/Care-Coordination.asp#RFS>
- Navigate to the link to the RFS form at the bottom of the section

Medical Records and Documents Requirements

Medical Records and documentation are required for all provider services. Providers are required to submit Medical documentation directly to the authorizing VAMC, preferably via upload to HSRM.

Billing and Other Referral Information**Submitting Claims**

ANY CLAIMS RELATED TO THIS EPISODE OF CARE MUST BE SUBMITTED TO TRIWEST OR DELTA DENTAL AS APPROPRIATE AND INCLUDE THE APPROVED REFERRAL NUMBER.**Methods to submit claims:****Electronic Data Interchange (EDI):**

Payer ID for Medical – TWVACCN

Payer ID for Dental – CDCA1

More information on how to submit claims can be found by visiting

<https://www.va.gov/COMMUNITYCARE/revenue-ops/Veteran-Care-Claims.asp>**Precertification**

The Standardized Episode of Care (SEOC) referral you have accepted does not include services that require third-party payer (TPP) precertification.

Community Care Medical Policies

VA Community Care Medical Policies describe standard VA health care benefits for services and procedures that community providers may recommend as necessary for a Veteran. Prior to providing care, providers should use the Community Care Medical Policies as a reference when determining if a Veteran meets VA clinical criteria. When additional services are requested, Community Care Medical Policies will be used to determine approval by a clinical reviewer. Community Care Medical Policies and supporting information can be found at:

<https://www.va.gov/COMMUNITYCARE/providers/info-CDI.asp>**Medical Devices**

As part of furnishing the services authorized on the approved referral, you may recommend that medical devices, adaptive equipment, or other items be provided for the treatment or rehabilitation of the Veteran's medical condition. In such instances, you must complete a Request for Services (RFS) as outlined above.

<https://www.va.gov/COMMUNITYCARE/providers/Care-Coordination.asp#RFS>

The RFS must (1) indicate that the condition for which the item is being prescribed is within the scope of the authorized services on the approved referral, (2) describe the item or service being prescribed with as much specificity as possible (including manufacturer and model, needed custom measurements, size), and (3) provide a brief but thorough plain-language explanation of how the item or service will serve the treatment or rehabilitation needs of the Veteran. All parts of the RFS form must be filled out completely.

VA staff will determine whether the prescribed item or service is one that VA is authorized to purchase. If the item is not something that we are authorized to purchase, VA staff will work with you to identify an alternative that will best meet the Veteran's clinical needs. In making this determination, VA staff will consider your clinical judgment and expertise and work with you to resolve outstanding issues quickly.

In the event of an urgent or emergent need, you may provide medical devices, necessary items or services prescribed to Veterans receiving care in the community for urgent or emergent conditions at the time of healthcare service delivery or soon thereafter. Urgent or emergent DME or Medical Devices may include, but are not limited to splints, crutches, canes, slings, soft collars, walkers, and manual wheelchairs. DME dispensed directly to the Veteran based on an urgent or emergent need will be paid by the CCN Third Party Administrator. Claims for these items should be submitted using the process outlined in the **Submitting Claims** section of this form.

Pharmacy**Drug Safety and Administration Requirements**

· Must follow the VA National Protocol and clinical guidance for Esketamine or Ketamine administration. Ketamine treatments for mental health or for pain are not approved under a CCN referral. Prior to administration it is required the ordering and administering providers review the VA Protocol and Clinical Guidance found through the VA Formulary Search Tool available here:

· <https://www.pbm.va.gov/apps/VANationalFormulary/>

Must follow the VA Opioid Safety Guidelines and complete the required Opioid Safety Training found here:

- <https://www.pbm.va.gov/apps/VANationalFormulary/>
- <https://www.va.gov/COMMUNITYCARE/providers/EDU-Training.asp>

CVS Caremark is the retail pharmacy network for Veterans' immediately needed or Urgent/Emergent prescriptions.

Immediate need prescriptions:

- Must follow the VA Urgent/Emergent Formulary which can be found at <http://www.pbm.va.gov/PBM/nationalformulary.asp>
- Prescription can only go up to a 14-day supply. No refills of the immediate need medication may be authorized.
- Only a seven-day supply for opioids, or up to the opioid prescribing limit allowed by State—whichever is less—may be authorized.

Immediate need prescription extending past 14 days:

- The provider will need to send second prescription (beyond 14 days) to the referring VA medical facility's pharmacy for prescription fulfillment services.

Routine/maintenance prescriptions:

- Must be sent to the referring VA medical facility's pharmacy

If you do not have the ability to electronically submit prescriptions to pharmacies, please contact the Community Care representative at the referring VA medical facility for their pharmacy fax number. Please refer to:

<https://www.va.gov/COMMUNITYCARE/providers/Service-Requirements.asp> for additional instructions related to prescriptions.

Clinical Information on the VA Order

Reason for Request:

Provider to receive results:hohlt

Type of Service: Evaluation and Treatment

Chief Complaint:

He does have recurrent skin cancer. He was diagnosed with Basosquamous carcinoma of skin lower extremity several years ago. I believe he has the same 1 going on on his lower abdomen.

Consult community care dermatology, preferably in Victoria versus El Campo Texas. Mass

Patient History / Clinical Findings / Diagnosis (Co-Morbidities):

n/a

Is this referral for Service Connected condition? No

Per VA PBM Formulary Process Management Policy from VHA Handbook 1108.08, the

use of drugs solely to improve normal physiologic function or to enhance body

appearance is generally not considered medically necessary and therefore are not

to be prescribed. Medications used for this purpose are considered cosmetic.

The use of medications for hair re-growth (e.g., minoxidil, finasteride, or

pimecrolimus) are considered cosmetic in nature and should not be prescribed

unless care is being provided in connection with the treatment of a

service-
connected injury.

**This 10-7080 - Approved Referral For Medical Care was generated on 04/23/2024 changes made to the referral after this date are not reflected on the form.



U.S. Department of Veterans Affairs

Veterans Health Administration
Office of Community Care

Health Share Referral Manager (HSRM): VA's Secure Online Portal for Managing Referrals and Authorizations

Why are community providers across the country excited to use HSRM?

- ◆ **Facilitates Health Information Exchange (HIE)** between community providers and VA via a unified platform
- ◆ Conveniently **organizes all active referrals** in one centralized location
- ◆ **Reduces time spent** waiting for fax, phone or email contact
- ◆ Allows community providers to **request authorization** for additional services or additional time to provide services
- ◆ Promotes **reduced turnaround time** for authorizations and reimbursement

HSRM Account Creation For Community Providers

- ◆ **STEP 1: Training:** Each staff member attends virtual training, completes eLearning lessons, or reviews one of the training guides
- ◆ **STEP 2: ID.me:** Each staff member creates an ID.me account and verifies their identity at <https://www.id.me/>
- ◆ **STEP 3: Submit EUT:** One facility point of contact (POC) fills out the End User Tracker (EUT), then sends it to HSRMSupport@va.gov
- ◆ **STEP 4: Receive Accounts:** Help Desk creates accounts in HSRM, then provides confirmation of account creation to the facility POC
- ◆ **STEP 5: Log Into HSRM:** Each staff member logs into HSRM at <https://ccracommunity.va.gov>
- ◆ Once these steps are complete, contact the VA Medical Center(s) you work with to let them know you have access to HSRM and to discuss your transition to HSRM

HSRM Registration Resources

- ◆ HSRM Support Points of Contact: <https://www.va.gov/COMMUNITYCARE/providers/HSRM-POC-List.asp>
- ◆ ID.me Registration Instructions: https://www.va.gov/COMMUNITYCARE/docs/pubfiles/factsheets/FactSheet_26-06.pdf#
- ◆ End User Tracker
 - End User Tracker Template: https://www.va.gov/COMMUNITYCARE/docs/providers/HSRM_End_User_Tracker.xlsx#
 - Send completed End User Tracker to HSRMSupport@va.gov
- ◆ HSRM Login: <https://ccracommunity.va.gov>

HSRM Educational Resources

- ◆ Live Virtual Training: https://www.train.org/virginia/course/1082953/live_event; please visit link to find available dates and to register in advance
- ◆ HSRM eLearning lessons: Log into <https://www.train.org/vha/welcome>, then search the course catalogue for "HealthShare Referral Manager"
- ◆ HSRM User Guide: https://www.va.gov/COMMUNITYCARE/docs/providers/HSRM_User_Guide.pdf
- ◆ HSRM Quick Reference Guide (QRG): https://www.va.gov/COMMUNITYCARE/docs/providers/HSRM_Quick_Reference_Guide.pdf#
- ◆ Additional information about HSRM: https://www.va.gov/COMMUNITYCARE/providers/Care_Coordination.asp
- ◆ HSRM Help Desk
Phone: 1-844-293-2272, Email: HSRMSupport@va.gov, Hours: Monday - Friday 7:30 a.m. - 5 p.m. Eastern Time

**Veterans Health
Administration****VHA Medical
Documentation**

Document Created: 04/23/2024 19:44

The documents accompanying this transmission contain confidential health information that is legally privileged. This information is intended only for the use of the individual or entity providing care to this Veteran. The authorized recipient of this information is prohibited from disclosing this information to any other party unless required to do so by law or regulation and is required to destroy the information after its stated need has been fulfilled and in accordance with agency destruction and retention requirements. If you are not the intended recipient, you are hereby notified that any disclosure, copying, or distribution is strictly prohibited as this information is protected by Federal Privacy law (e.g., HIPAA Privacy Rule). If you have received this information in error, please notify the sender immediately and arrange for the return or destruction of these documents.

AUDIE L MURPHY MEMORIAL VAMC 7400 Merton Minter Blvd, San Antonio, Texas 78229-4404
VA Referrals phone: 210-949-3850 Fax:

Point of Contact (POC): MARTINEZ, GRETCHEN
POC Phone: 2109493850 Fax:

Patient Name: DOCKINS,FLOYD
NORMAN

DOB: 01/27/1950

38 TERRY STREET
PORT LAVACA, TEXAS 77979

Phone (mobile): (928)830-3524

Referral Type: Community Care
Network

Next of Kin contact information

DOCKINS,CYNTHIA

Address (Next Of Kin)
38 TERRY STREET
PORT LAVACA, TEXAS 77979

Phone: (928)830-3524
Phone (work): NONE

CONSULTS

Current PC Provider: HOHLT,RUSSELL W
Current PC Team: VICTORIA BLUE TEAM *WH*
Current Pat. Status: Outpatient
Primary Eligibility: NSC
UCID: 671_8545155
Patient Type:
OEF/OIF: NO

Order Information

To Service: COMMUNITY CARE-DERMATOLOGY
Attention:
From Service: VOP PACT PC BLUE
Requesting Provider: HOHLT,RUSSELL W
Service is to be rendered on an OUTPATIENT basis
Place: Consultant's choice
Urgency: Routine

Clinically Ind. Date: 2024-04-25 00:00:00

DST ID:

Orderable Item:

Consult: Consult

Provisional Diagnosis: Localized Swelling, Mass And Lump, Trunk (ICD-10-CM R22.2)

Reason For Request:

Provider to receive results:hohlt

Type of Service: Evaluation and Treatment

Chief Complaint: He does have recurrent skin cancer. He was diagnosed with Basosquamous carcinoma of skin lower extremity several years ago. I believe he has the same 1 going on on his lower abdomen. Consult community care dermatology, preferably in Victoria versus El Campo Texas. Mass

Patient History / Clinical Findings / Diagnosis (Co-Morbidities):

n/a

Is this referral for Service Connected condition? No

Per VA PBM Formulary Process Management Policy from VHA Handbook 1108.08,

the use of drugs solely to improve normal physiologic function or to enhance body appearance is generally not considered medically necessary and therefore are not to be prescribed. Medications used for this purpose are considered cosmetic. The use of medications for hair re-growth (e.g., minoxidil, finasteride, or pimecrolimus) are considered cosmetic in nature and should not be prescribed unless care is being provided in connection with the treatment of a service- connected injury.

Inter-facility Information

AUDIE L. MURPHY MEMORIAL HOSP

Status: ACTIVE

Last Action: ADDED COMMENT

Facility

Activity Date/Time/Zone Responsible Person Entered By

CPRS RELEASED ORDER 04/04/24 11:11 HOHLT,RUSSELL W HOHLT,RUSSELL W
ADDED COMMENT 04/04/24 00:00 HOHLT,RUSSELL W HOHLT,RUSSELL W
RECEIVED 04/04/24 14:05 LARS,SUSAN M LARS,SUSAN M
ADDED COMMENT 04/04/24 14:07 LARS,SUSAN M LARS,SUSAN M

Note: TIME ZONE is local if not indicated

No local TIU results or Medicine results available for this consult

=====
===== END =====

PROBLEM LIST

Sensitive Diagnoses

No sensitive diagnoses were provided.

Other Diagnoses

Problem	Code
Actinic keratosis	L57.0
Carcinoma in situ of bladder	D09.0
Constipation, unspecified	K59.00
Essential (primary) hypertension	I10.

Gross hematuria	R31.0
Hypothyroidism, unspecified	E03.9
Localized swelling, mass and lump, trunk	R22.2
Mixed hyperlipidemia	E78.2
Neoplasm of uncertain behavior of skin	D48.5
Neoplasm of unsp behavior of bone, soft tissue, an	D49.2
Other disorders of lung	J98.4
Other seborrheic keratosis	L82.1
Presence of left artificial hip joint	Z96.642
Tobacco use	Z72.0
Type 2 diabetes mellitus with hyperglycemia	E11.65
Unspecified disorder of refraction	H52.7
Unspecified malignant neoplasm of skin, unspecifie	C44.90
Vertebrogenic low back pain	M54.51

MEDICATIONS

100 most recent outpatient medications released by VA to Veteran in the last 6 months

Medication Name and Dose	Quantity	Refill Number	Issue and Fill Date	Status
ALBUTEROL 90MCG (CFC-F) 200D ORAL INHL INHALE 2 PUFFS BY INHALATION EVERY 4 HOURS FOR BREATHING. *MAX OF 8 PUFFS PER 24 HOURS*	Qty:3	Fill: 1 of 3	Orig: 2024-04-04 Last: 2024-04-04	ACTIVE
LEVOTHYROXINE NA (SYNTHROID) 75MCG TAB TAKE ONE TABLET BY MOUTH EVERY MORNING BEFORE MEAL FOR HYPOTHYROIDISM	Qty:90	Fill: 1 of 3	Orig: 2024-04-04 Last: 2024-04-04	ACTIVE
NICOTINE 14MG/24HR PATCH APPLY ONE PATCH TO SKIN EVERY DAY FOR SMOKING CESSATION BEGIN AFTER LAST 21MG PATCH.	Qty:14	Fill: 1 of 0	Orig: 2024-04-04 Last: 2024-04-04	ACTIVE
NICOTINE 21MG/24HR PATCH APPLY ONE PATCH TO SKIN EVERY DAY FOR SMOKING CESSATION USE FOR 42 DAYS THEN START 14MG PATCH.	Qty:42	Fill: 1 of 0	Orig: 2024-04-04 Last: 2024-04-04	ACTIVE
NICOTINE 7MG/24HR PATCH APPLY ONE PATCH TO SKIN EVERY DAY FOR SMOKING CESSATION BEGIN AFTER LAST 14MG PATCH.	Qty:14	Fill: 1 of 0	Orig: 2024-04-04 Last: 2024-04-04	ACTIVE
CETIRIZINE HCL 10MG TAB TAKE ONE TABLET BY MOUTH EVERY DAY FOR ALLERGIES	Qty:60	Fill: 3 of 3	Orig: 2022-11-23 Last: 2023-10-24	EXPIRED
SILDENAFIL CITRATE 100MG TAB TAKE ONE-HALF TABLET BY MOUTH AS NEEDED 30 MINUTES PRIOR TO SEXUAL ACTIVITY	Qty:2	Fill: 6 of 11	Orig: 2022-11-23 Last: 2023-11-23	EXPIRED

NON-VA MEDICATIONS

Local Drug Name and Dose	Medication Route	Schedule
ASPIRIN 81MG EC TAB	MOUTH	EVERY DAY
ACETAMIN 300 MG W/COD 30 MG TAB	MOUTH	EVERY 8 HOURS AS NEEDED

ALLERGIES

Name	Origin	Verified
CODEINE	Origin: 2012-02-09	Verified: 2012-02-09

PROGRESS NOTES

LOCAL TITLE: VOPC PHYSICIAN NOTE
STANDARD TITLE: PHYSICIAN NOTE
DATE OF NOTE: APR 04, 2024@10:44 ENTRY DATE: APR 04, 2024@10:44:39
AUTHOR: HOHLT,RUSSELL W EXP COSIGNER:
URGENCY: STATUS: COMPLETED

DATE: APR 04, 2024

DOCKINS,FLOYD NORMAN, ***_**_****
Age: 74 Sex: MALE Race: WHITE

Weight: 136.7 lb [62.01 kg] (04/04/2024 10:23) Height: 64 in [162.6 cm]
(04/04/2024 10:23)
Respiration: 20 (04/04/2024 10:23) Temperature: 97.3 F [36.3 C] (04/04/2024
10:23)
Pulse: 87 (04/04/2024 10:23) Blood Pressure: 142/72 (04/04/2024 10:33)
Pain Scale: 0 (04/04/2024 10:23)

DISCLAIMER: For patients being seen at DOD, Cerner Sites, and VHA facilities,
Remote Medications, Remote allergies and adverse drug reactions (ADRs) may be
incomplete. Patient medication list and allergies must be viewed using Joint
Legacy Viewer (JLV). For patients seen at DOD, Cerner Sites and VHA
facilities,
I have reviewed patient medications using JLV.

ALLERGIES REMOTE AND LOCAL REVIEW:
FACILITY ALLERGY/ADR

No Remote Allergy/ADR Data available for this patient
AUDIE L. MURPHY MEMORIAL HOSP CODEINE

Remote and Local Allergies reviewed, and reactions confirmed.

Medications:
Active Inpatient, Outpatient and Clinic Medications (including Supplies):

Active Non-VA Medications	Start Date	Refills	Expiration
=====			
1) Non-VA ACETAMIN 300 MG W/COD 30 MG TAB	ACTIVE		
Sig: 1 TABLET MOUTH EVERY 8 HOURS AS NEEDED			
2) Non-VA ASPIRIN 81MG EC TAB	ACTIVE		
Sig: 81MG MOUTH EVERY DAY			

Medication Reconciliation:

I have performed med rec with the patient/caregiver utilizing the Essential
Medication List for Review (EMLR) MRT1. This included, under MRR1 active and

pending prescriptions dispensed from local and remote VA prescriptions, DoD prescriptions, local non-VA medications, local inpatient medications orders, clinic medication orders and prescriptions that have expired or been discontinued in the past 90 days.

=====

MEDICATION REVIEW

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No changes made to medications during this visit.

Problem List:

Active problems - Computerized Problem List is the source for the following:

1. Back pain
2. Constipated
3. Malignant neoplasm of urinary bladder
4. Basosquamous carcinoma of skin
5. Actinic keratosis
6. Seborrheic keratosis
7. Neoplasm of uncertain behavior of skin of lower limb
8. Refractive error
9. Multiple nodules of lung
10. Tobacco use
11. Acquired hypothyroidism
12. Osteoarthritis of left hip joint (SNOMED CT 323291000119108)

LABS:

CBC:

Collection DT	Specimen	Test Name	Result	Units	Ref
Range					
03/28/2024 10:06	BLOOD	WBC	7.9	10.e3/uL	4.0 - 10.0
03/28/2024 10:06	BLOOD	RBC	4.36 L	10.e6/uL	4.5 - 5.9
03/28/2024 10:06	BLOOD	HGB	14.1	g/dL	13.5 - 17.5
03/28/2024 10:06	BLOOD	HCT	42.5	%	41.0 - 53.0
03/28/2024 10:06	BLOOD	MCV	97.3	fL	78.0 - 98.0
03/28/2024 10:06	BLOOD	MCH	32.3	pg	26.0 - 34.0
03/28/2024 10:06	BLOOD	MCHC	33.2	gm/dL	32.0 - 36.0
03/28/2024 10:06	BLOOD	PLTS	278	10.e3/uL	150 - 400

Chem 20:

Date	Test	Result	Units	Normal Range	Specimen
03/28/2024	CREATININE	0.9	mg/dL	.7 - 1.3	PLASMA
03/28/2024	UREA NITROGEN	8	mg/dL	7 - 25	PLASMA
03/28/2024	GLUCOSE	118 H	mg/dL	70 - 105	PLASMA
03/28/2024	SODIUM	139	mmol/L	136 - 145	PLASMA
03/28/2024	POTASSIUM	4.9	mmol/L	3.5 - 5.1	PLASMA
03/28/2024	CHLORIDE	102	mmol/L	98 - 107	PLASMA
03/28/2024	CALCIUM	10.2	mg/dL	8.6 - 10.3	PLASMA
03/28/2024	PROTEIN,TOTAL	7.5	g/dL	6.4 - 8.9	SERUM
03/28/2024	ALBUMIN	4.4	g/dl	3.5 - 5.7	SERUM
03/28/2024	TOT. BILIRUBIN	0.3		0.3 - 1.0	SERUM

03/28/2024 ALKALINE PHOSP 63 U/L 34 - 104 SERUM
 03/28/2024 AST 14 IU/L 13 - 39 SERUM
 03/28/2024 ALT 15 IU/L 7 - 52 SERUM
 03/28/2024 ANION GAP 10.0 mmol/L PLASMA
 03/28/2024 CO2 27.0 mmol/L 21 - 31 PLASMA
 03/28/2024 ICTERUS 0 NEG PLASMA
 03/28/2024 LIPEMIA 0 NEG PLASMA
 03/28/2024 HEMOLYSIS 0 NEG PLASMA
 03/28/2024 OSMOLALITY-CAL 277.00 mOsm/kg PLASMA
 01/31/2022 zzEGFR >60 PLASMA
 03/28/2024 EGFR CKD EPI 90 PLASMA

A1C:

Date	Test	Result	Units	Normal Range	Specimen
03/28/2024	HEMOGLOBIN A1C	6.2 H	%	4.0 - 6.0	BLOOD

PROSTATIC SPECIFIC ANTIGEN 9/1/23 10:45 0.49

LIPID PROFILE Coll. dat3/28/24 10:06 9/1/23 10:45 3/2/23 10:58

CHOL	180	186	178
TRIG	188	221 H	195
HDL	42	41	40
LDL CHOLESTEROL	100.00	101.00	99.00
VLDL CHOL	38.0	44.0	39.0
LDL DIRECT			

Chief complaint

-vet will bring bottles of meds he is currently taking

-vet wanting med for tob cessation, states he will discuss with PCP what med is best for tob cessation

-vet c/o crusted area to his lower abdomen, onset 3 months, states he is not sure if it is an ingrown hair, states he tried to remove it and it made area worse

-vet brought med bottles of current meds he is taking:

albuterol inhaler 2 puffs as needed
 levothyroxine 75 mcg 1 tab daily
 otc Sinex 1 spray each nostril as needed

-states he is currently not taking ASA at this time and is no longer taking acetaminophen/cod, states he is no longer taking polyethylene glycol powder

This is a 6-month follow-up appointment.

Subjective

This patient has bladder cancer and just finished his chemotherapy and radiation (2022), history essential hypertension, dyslipidemia, history of diabetes type 2, chronic lower back pain, bilateral hip pain, smoking disorder, refused to take a statin who is taking all of his medications without side effects. He is taking his medication without any side effects.

I reviewed his chief plan above.

He went all of his medications through the VA.

Is patient has a nodule, lower abdomen. He said he suddenly had it 3 to 4 weeks ago, and improved greatly since that time. He did have basosquamous carcinoma on his lower extremity. By exam, he had a jagged edged long narrow head, almost white with a lot of pain when I palpated.

Blood pressure is slightly elevated today. However I reviewed his other vitals from 6 months ago and 1 year ago. It is always normal. I will monitor.

I did answer all of his questions.

Review of systems

No chest pain, dyspnea, abdominal pain, nausea, vomiting, fever, chills, dizziness, lightheaded, sore throat, cough, diarrhea, constipation

Physical exam

General the patient is alert and oriented x4

Psych no suicidal or homicidal thoughts

Neck is supple no cervical lymphadenopathy

Lungs clear to auscultation bilaterally no rhonchi or wheezes

Heart normal rate normal rhythm S1-2 heard

Abdomen normal bowel sounds at all 4 quadrants, no rebound, tenderness, guarding.

Extremities show no cyanosis no clubbing and no edema

Skin warm and dry

Gait normal

Skin nodule lower abdomen just below the navel area, underneath the belt a jagged edged long narrow head, almost white with a lot of pain when I palpated.

I reviewed his labs with him today.

Assessment and plan

Skin nodule loratadine, see subjective and physical exam above. Consult dermatology in Victoria Texas versus El Campo Texas.

Transitional cell carcinoma continue community care urologist and oncologist.
Stable

History of hypertension and he is no longer taking hypertensive medications. He does no longer have diabetes (9/1/2023 A1 6.5) as well.

Chronic lower back pain. stable

Chronic bilateral hip pain. stable

Continue smoke cigarettes daily. I did counsel him. He will take nicotine patch, highest dos. I did counsel him. Continue take albuterol inhaler 1-2 p.o. per nostril every 4 hours as necessary for wheezing/shortness of breath.

Constipation continue MiraLAX 17 g powder daily.

Hypothyroidism continue take levothyroxine 75mcg daily. Stable

Disposition:

I have reviewed recent test results at length with patient. I did give him the opportunity to assess questions, and answered all of his questions with satisfaction. Patient verbalized understanding of all information and instructions.

I have instructed patient to bring all of his medication bottles at the next appointment for complete medication reconciliation. Patient should include the OTC medications as well as prescribed medication.

100% of this time was spent evaluating this patient, as well as counseling and coordinating the care addressing conditions, and she about the course of the disease, answering questions, making recommendations, prescribing medications with appropriate and entered the necessary referrals to optimize care, as necessary.

DC home

continue medications, as prescribed.

RETURN TO CLINIC

- 6 months with PCP and repeat labs CBC, Chem-12, hemoglobin A1c, TSH, lipid profile, urinalysis

Dragon Speak Clarification: Dragon Speak dictation program was used by this author for this note. Attempts at proofreading are not always successful at identifying errors and word transposition may occur. Please contact writer if significant errors are present.

/es/ RUSSELL W HOHLT
MD, Chief Medical Officer
Signed: 04/04/2024 11:21

Department of Veterans Affairs		REQUEST FOR SERVICES (RFS) FORM	
PREVIOUS AUTHORIZATION NUMBER:		NOTE: The Request for Services (RFS) Form 10-10172 must be submitted via an approved method (HSRM, Electronic fax, Direct Messaging, traditional fax, or mail). Completion of this form is REQUIRED and MUST BE SIGNED by the requesting provider for further care to be rendered to a Veteran patient.	
TODAY'S DATE (MM/DD/YYYY):			
SECTION I: VETERAN INFORMATION			
1. VETERAN'S LEGAL FULL NAME (First, MI, Last):			2. DOB (MM/DD/YYYY):
3. VA FACILITY: STVHCS/Pink Team - Team Fax: (210) 443-0301		4. VA LOCATION: San Antonio, TX Ph: (210) 949-3850	
SECTION II: ORDERING PROVIDER INFORMATION			
5. REQUESTING PROVIDER'S NAME:		6. NPI #:	7. SPECIALTY:
8. OFFICE NAME & ADDRESS:			
9. SECURE EMAIL ADDRESS:			
10. PHONE NUMBER:		11. FAX NUMBER:	
☐ 12. INDIAN HEALTH SERVICES (IHS) PROVIDER?			
SECTION III: TYPE OF CARE REQUEST			
13. PLEASE INDICATE CLINICAL URGENCY (Urgent care is only applicable for requests that require less than 3 days to process. If care is needed within 48 hours or if Veteran is at risk for Suicide/Homicide, please call the VA directly on the same day as completed RFS form submission. Do NOT mark urgent for administrative urgency): ☐ ROUTINE ☐ URGENT			
14. DIAGNOSIS (ICD-10 Code/Description):		15. DATE OF SERVICE (MM/DD/YYYY) &/OR ANTICIPATED LENGTH OF CARE:	
16. CPT/HCPCS CODE &/OR DESCRIPTION OF REQUESTED SERVICES (Include units/visits, add second list page, if needed):			
17. HOW MANY VISITS HAVE OCCURRED SO FAR? (If known)		18. IS THIS A REFERRAL TO ANOTHER SPECIALTY? ☐ YES (If "YES," please fill out the Servicing Provider/Specialty information below) ☐ NO	
19. SERVICING PROVIDER'S NAME:		20. NPI #:	21. SPECIALTY:
22. OFFICE NAME & ADDRESS:			
23. SECURE EMAIL ADDRESS:			
24. PHONE NUMBER:		25. FAX NUMBER:	
SECTION IV: TYPE OF SERVICE REQUESTED			
26. OUTPATIENT CARE: ☐ PT ☐ OT ☐ SPEECH THERAPY FREQUENCY & DURATION:		27. SURGICAL PROCEDURE: ☐ INPATIENT ☐ OUTPATIENT FACILITY NAME:	
28. ☐ IN-OFFICE PROCEDURE		29. INPATIENT CARE: ☐ LTACH ☐ ACUTE REHAB ☐ BH	
30. ☐ ADDITIONAL OFFICE VISITS (List # needed):		31. ☐ EXTENSION OF VALIDITY DATES	
32. ☐ EMERGENCY ROOM CARE		33. ☐ LABS (If done outside of office, please provide facility above)	
34. ☐ RADIOLOGY/IMAGING (If done outside of office, please provide facility above)		35. ☐ PRE-OPS LABS ☐ CHEST XRAY ☐ EKG ☐ OTHER:	
36. JUSTIFICATION FOR REQUEST (To avoid delays in care, include appropriate documentation such as office notes, current treatment plans, clinical history, laboratory results, radiology results &/or medications to support the medical necessity of services requested).			

VETERAN'S LEGAL FULL NAME (First, MI, Last):		
SECTION V: GERIATRICS AND EXTENDED CARE SERVICES (If applicable)		
37. ☐ COMMUNITY ADULT DAY HEALTH CARE ☐ COMMUNITY NURSING HOME ☐ HOMEMAKER/HOME HEALTH AIDE ☐ HOME INFUSION ☐ HOSPICE/PALLIATIVE CARE ☐ RESPITE ☐ SKILLED HOME HEALTH CARE ☐ OTHER: _____		
FREQUENCY & DURATION: _____		
38. JUSTIFICATION FOR REQUEST (To avoid delays in care, include appropriate documentation such as office notes, current treatment plans, clinical history, laboratory results, radiology results &/or medications to support the medical necessity of services requested).		
SECTION VI: HOME OXYGEN INFORMATION (If applicable)		
39. PAO2 AT REST:	40. O2 SAT AT REST:	41. OXYGEN FLOW RATE:
42. EXTENT OF SUPPORT (Continuous, Intermittent, Specific Activity):		
43. OXYGEN EQUIPMENT (Stationary/Portable):		
44. DELIVERY SYSTEM (Cannula, Mask, Other):		
SECTION VII: DME & PROSTHETICS INFORMATION (If applicable)		
45. HCPS FOR THE ITEM(S) BEING PRESCRIBED:		
46. BRAND, MAKE, MODEL, PART NUMBERS:		
47. MEASUREMENTS:		
48. QUANTITY:	49. ICD-10:	50. PROVISIONAL DIAGNOSIS:
51. DELIVERY/PICKUP OPTIONS: ☐ DELIVER TO ORDERING PROVIDER'S ADDRESS ☐ VETERAN WILL PICKUP AT THE VA MEDICAL CENTER ☐ DELIVER TO COMMUNITY VENDOR FOR DELIVERY & SETUP FOR DME ☐ DELIVER TO VETERAN'S HOME		
SECTION VIII: DURABLE MEDICAL EQUIPMENT (DME) EDUCATION & TRAINING (If applicable)		
Please see DME Requirements/Pharmacy Requirements - Community Care (va.gov) for URGENT DME requests.		
NOTE: Failure to thoroughly complete the RFS for DME will result in delayed patient care & prevent the VA from DME fulfillment.		
52. BEFORE DME WILL BE ISSUED, EDUCATION, TRAINING, &/OR FITTING OF DME (as applicable for the specific DME being ordered) TO THE VETERAN MUST BE COMPLETE. PLEASE INDICATE WHETHER THE FOLLOWING HAS BEEN COMPLETED FOR THE VETERAN. NOTE: If not completed, DME will be mailed to requesting provider's address to coordinate an alternative time for proper instruction on DME use.		A. EDUCATION: ☐ YES ☐ NO B. TRAINING: ☐ YES ☐ NO ☐ N/A C. FITTING: ☐ YES ☐ NO ☐ N/A
53. JUSTIFICATION FOR REQUEST (To avoid delays in care, include appropriate documentation such as office notes, current treatment plans, clinical history, laboratory results, radiology results &/or medications to support the medical necessity of services requested).		

VETERAN'S LEGAL FULL NAME (First, MI, Last):	
SECTION IX: THERAPEUTIC FOOTWEAR ASSESSMENT INFORMATION (If applicable)	
54. FILL OUT THE INFORMATION BELOW (If applicable): ☐ LEFT FOOT ☐ RIGHT FOOT ☐ BILATERAL ☐ PREFABRICATED THERAPEUTIC FOOTWEAR ☐ CUSTOM THERAPEUTIC FOOTWEAR NOTE: For prescription of therapeutic footwear for severe or gross foot deformity which cannot be accommodated with conventional footwear. DESCRIBE FOOT DEFORMITY AND ADDITIONAL DETAILS:	NOTE: For prescription of therapeutic footwear due to disease pathology resulting in neuropathy or peripheral artery disease. 55. CHECK APPROPRIATE DIABETIC/AMPUTATION RISK SCORE: ☐ RISK SCORE 2: PATIENT DEMONSTRATED SENSORY LOSS (<i>inability to perceive the Semmes-Weinstein 5.07 monofilament</i>), DIMINISHED CIRCULATION AS EVIDENCED BY ABSENT OR WEAKLY PALPABLE PULSES, FOOT DEFORMITY, OR MINOR FOOT INFECTION, & A DIAGNOSIS OF DIABETES. ☐ RISK SCORE 3: PATIENT DEMONSTRATED PERIPHERAL NEUROPATHY WITH SENSORY LOSS (<i>i.e., inability to perceive the Semmes-Weinstein 5.07 monofilament</i>), AND DIMINISHED CIRCULATION, AND FOOT DEFORMITY, OR MINOR FOOT INFECTION & A DIAGNOSIS OF DIABETES, OR ANY OF THE FOLLOWING BY ITSELF: (1) PRIOR ULCER, OSTEOMYELITIS OR HISTORY OF PRIOR AMPUTATION; (2) SEVERE PERIPHERAL VASCULAR DISEASE (PVD) (<i>intermittent claudication, dependent rubor with pallor on elevation, or critical limb ischemia manifested by rest pain, ulceration or gangrene</i>); (3) CHARCOT'S JOINT DISEASE WITH FOOT DEFORMITY; & (4) END STAGE RENAL DISEASE. NOTE: Only patients who are experiencing medical conditions noted in the risk scores can be prescribed therapeutic/diabetic footwear.
*ATTESTATION: I do hereby attest that the forgoing information is true, accurate, & complete to the best of my knowledge & I understand that any falsification, omission, or concealment of material fact may subject me to administrative, civil, or criminal liability. I do hereby acknowledge that VA reserves the right to perform the requested service(s) if the following criteria are met: (1) The patient agrees to receive services from VA (2) Service(s) are available at VA facility & are able to be provided by the clinically indicated date (3) It is determined to be within the patients best interest. Upon completion of the requested service(s), VA will provide all resulting medical documentation to the ordering provider. If all criteria listed are not true & VA agrees the service(s) are clinically indicated, VA will provide a referral for services to be performed in the community. I do hereby attest that upon receipt of order/consult results, I will assume responsibility for reviewing said results, addressing significant findings, & providing continued care.	
56. REQUESTING PROVIDER SIGNATURE (Required):	57. TODAY'S DATE (MM/DD/YYYY):

To facilitate timely review of this request, the most recent office notes & plan of care must accompany this signed form.

For more information please visit: https://www.va.gov/COMMUNITYCARE/providers/Care_Coordination.asp.

VA Community Care Medical Policies describe standard VA health care benefit for services and procedures that community providers may recommend as necessary for a Veteran. Prior to providing care, providers should use the Community Care Medical Policies as a reference when determining if a Veteran meets VA clinical criteria. When additional services are requested, Community Care Medical Policies will be used to determine approval by a clinical reviewer. Community Care Medical Policies & supporting information can be found at: <https://www.va.gov/COMMUNITYCARE/providers/Medical-Policy.asp>